

Dependence Theory: Distinguishing Dependence From Pathological Use

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Abstract

Addiction is often associated with the term “dependence.” In what follows, I explain that this association is a conflation that has caused serious problems, even contradictions, in how dependence has been characterized both socially and clinically. I show precisely how an evaluatively negative interpretation of dependence—one focused on subjugation and compulsion—misinformed addiction. Rather than being synonymous, I explain why current philosophical literature on addiction understands pathological use as instead contrasting dependence. Specifically, I use the phrase “*persistent unsuccessful dependence*” to describe the current model. Under this model of addiction, the ailment of pathological use would not be a case of too much dependence, but rather a case of having too much unsuccessful dependence. By transitioning from an evaluatively negative interpretation of dependence to an evaluatively neutral, and teleological, interpretation of it, I show how we can both capture intuitive cases of addiction and solve puzzling cases. Indeed, the goal of implementing this framework is to increase the accuracy of addiction diagnosis.

Keywords: addiction, dependence, psychiatry, diagnosis

1. Introduction

We hear the phrase “alcohol-dependent” and think: *addicted to alcohol*. We hear “dependent on drugs” and think: *drug-addict*. There is a sense of “dependence” that just means “addiction.”¹ Here, I show that this is a conflation that has caused serious problems in how dependence is

¹ I am aware that “dependence” is also a technical term in the metaphysical literature on grounding, but I am referring to a different use here.

characterized both socially and clinically. The ultimate worry is that interpretation misunderstands and misdiagnoses what addiction is due to a misguided understanding of “dependency.” I will show that an alternative analysis of dependence that makes it clear that dependence and addiction are distinct in ways that are salient to clinical and psychiatric contexts.

Consider the following anecdote. If we look back at the last 15 years of technological development, one might be able to recall a certain hesitance. Indeed, there were people disturbed by, say, the sudden widespread onset of cell-phones. The phrase, “You kids are so dependent on your phones,” still rings in my mind. Or “You kids are addicted to your cell-phones.” To many Western ears, the transition from talking about dependence to talking about addiction is smooth and understandable. This paper shows why philosophers of psychiatry question the logic of that transition.

The word “addiction” has a common interpretation. Oftentimes, people translate “addiction” to mean some version of: *You couldn't stop even if you wanted to*. However, that is not what current philosophical literature on addiction suggests. The last 25 years have seen a paradigm-shift in the addiction literature. Current theoretical and empirical research on pathological use suggests that addicts suffer from distorted evaluations, not failures in volitional capacities—a big shift for the addiction literature. In order to reflect this shift properly, I here explicitly explain why we have changed how we characterize addiction's relation to dependence in the philosophical and psychiatric literature.

To be clear, I will be focusing on addiction as a pathology.² That is, I will be characterizing addiction as something that should be treated. Note that this is a methodological choice, not a metaphysical claim. If there is a form of addiction that does not need treatment—a form which may exist—one would need to characterize it differently than I do here. The characterization of addiction I offer here describes addiction as a problem, as something whose absence would be beneficial. To emphasize this point, I will forefront the conversation by pointing to a common character in philosophical literature on addiction—‘the willing-addict.’ That is, in section two, I will explain how my view of addiction argues that the willing addict is not a pathological user. Under my model of pathological use, it is contradictory to be a willing addict.

In the following section, section three, I begin my argument. I will first provide a preliminary analysis of the history of the “dependence” concept in general, and in the Diagnostic and Statistical Manual of Mental Disorders (DSM) specifically. Briefly, dependence is understood as an evaluatively negative relation where the dependent is somehow subordinate to, or compelled by, the entity depended on. Then I motivate the problem by examining the issues that arise from misdiagnosing addiction.

²I am going to use the word “addiction” interchangeably with “substance use disorder” because, for my purposes, they are capturing the same phenomenon—namely, pathological use. The DSM-5-TR has abandoned the word “addiction” because it has caused confusion between its clinical uses and society’s evaluative uptake. When I say “addiction,” I am referring to what the DSM-5-TR calls “substance use disorder” or any “-use disorder” including behavioral disorders such as gambling disorder.

In section four, I will describe two characterizations of addiction that are based on different interpretations of the word “compulsive”—the irresistibility model and the akratic model. I will argue that the irresistibility model relies on the negative interpretation of dependence described in the previous section, and falls apart. We have seen this model fall apart in other literature as well, as in philosopher Hanna Pickard’s (2018) *Puzzle of Addiction*. Then I will argue that the akratic model is poorly described by the term “compulsive,” being better understood by a departure from one’s values. I incorporate Pickard’s work, and those like it, to corroborate the notion that compulsion is a poor criteria for addiction diagnosis.

In section five, I will propose an evaluatively neutral analysis of dependence that better captures what ‘goes wrong’ in addiction. Briefly, my positive argument is as follows: *Agent A depends on X to achieve goal G*. From there, I argue that, when we are talking about agents, dependencies may either succeed or fail. A dependence is successful when an agent (A) depends on X to achieve G, actually achieves G through X, and there are no unacceptable consequences. Common examples would be: drinking water for hydration, wearing shoes to protect one’s feet, or using an EpiPen to prevent anaphylaxis. Unsuccessful dependence, by contrast, is failure either in the main goal pursuit or in the acceptability of the consequences. Reasonable examples, respectively, could be: depending on Ibuprofen to cure viral infections, or depending on a knife to cut through carrots but cutting through one’s finger as well. I argue that this more evaluatively neutral understanding of dependence in lieu of evaluatively negative uses—in discussions of addiction—improves our understanding of what counts as pathological use. In fact, it better aligns with contemporary philosophical and psychiatric addiction literature. I will conclude by suggesting that addiction is currently best understood, not as similar to dependence generally, but as persistent unsuccessful dependence (PUD).

2. The Willing Addict?

Here is a classic puzzle in the addiction literature that my view solves differently than the dependence-based view. The question goes: Would the willing addict benefit from addiction treatment? Beginning with philosopher Harry Frankfurt, the “willing addict” is a character that appears relatively frequently in philosophical literature on addiction. Briefly, the willing addict is an addict who finds her addiction acceptable. I like how philosopher Edmund Henden puts it:

Not all addicts are ambivalent, vacillating, or conflicted about their drug use. Imagine, for example, a non-addicted recreational smoker who genuinely values smoking, perhaps because she associates it with a cool lifestyle. Suppose her smoking escalates, and becomes an addiction. Although this means her smoking becomes excessive, she may have no desire to quit. Her long-term perspective and value system remain the same throughout and beyond this transitional phase, and she regulates her life in accordance with her values. (Pickard & Ahmed, 2019).

What I like about Henden’s description here is his emphasis on the coherent value structures that define the willing addict. She is “in accordance with her values.” That is what makes the puzzle a puzzle. Indeed, it often feels counterintuitive, paternalistic in fact, to “treat” someone who is aptly representing her values. The dependence-based view of addiction could argue that the willing addict is still a pathological user on account of her high level of dependence. I will argue that if one is doing what aligns with one’s values, then one is not an addict. So, in my view, the willing addict is a contradiction. It is a moot point whether they need treatment because, as I will

argue, addiction is necessarily value-conflicting. What we may find is that one who is both “willing” and arguably still an addict is one who is arguably self-deceived about her values (or how to achieve them).³

I will suggest that the reason the undeceived willing addict appeared viable in the first place is because we understood addiction through an evaluatively negative understanding of dependence. That is, we saw mere dependence—as would be the case for Henden’s understanding of a willing addict—as enough evidence to suggest that the user was using pathologically. However, my view suggests that the significance of a given cost, including losing “full control of her cigarette smoking,” must be contextualized around the goals of the individual in question (Pickard & Ahmed, 2019). Only then will we be able to know if the costs outweigh the benefits, which is, as Pickard (2018) says, the point at which use becomes pathological. Or at least it is when addiction treatment would appear beneficial.

In what follows, I argue that part of what made the willing addict a seemingly valid concept in the first place is our evaluatively negative understanding of dependence. It appears to many Americans that dependence itself is a problem, regardless of how the dependent herself feels. It is prudentially bad to be dependent. In the next section, I trace how this evaluatively negative understanding of dependence informed our understanding of addiction. With reference

³ While I do not have the space to go into satisfactory detail here, an example of this may be one who satisfies certain “adaptive preferences,” as defined by philosopher Serene Khader (2011). Briefly, Khader argues that one may endorse certain preferences that she developed in response to deprived circumstances, but then opt to reject these same preferences in otherwise flourishing circumstances. Can the satisfaction of an adaptive preference be a case of addiction? Further work is required to bridge these two literatures together.

to philosophers such as Hanna Pickard and Neil Levy, I will then dismantle it, and begin to build a view of pathological-use on a reformed, and contemporarily informed, understanding of dependence.

3. How Negative Views of Dependence Informed Our Understanding of Addiction

In contemporary American culture, “dependency” is most often evaluated negatively. More specifically, dependency is associated with: incompleteness, illegitimacy, degradation, subordination, low socio-economic status, and lack of willpower (Fraser and Gordon, 1994). As sociologists Fraser and Gordon write,

In preindustrial English usage, the most common meaning of *dependency* was subordination... A second icon of industrial dependency was... “the colonial native” and “the slave” who “personified political subjugation.” (Fraser and Gordon, 1994; original italics)

Dependence, construed to imply subjugation, is the kind of dependence at issue here. Likened to political subjugation, this notion of dependence is supposed to be understood as prudentially bad for the dependent because it limits one’s agency or autonomy.

Addiction too has come to be characterized by limitations in autonomy (see the next section for more details). We can see this characterization in the heavy association between dependence and addiction. For example, people suffering from alcohol-use disorders are often referred to as alcohol “dependent” (cf: Prashanth and Sowmaya, 2019; Gerhant and Olajossy, 2019; Mejlidal et al., 2020; Stojanović & Stojanović, 2020; Higuchi et al., 2020). There is also an

official scale—called “The Alcohol Dependence Scale”—that is used to evaluate addiction (Mejldal et al., 2020). The Diagnostic and Statistical Manual of Mental Disorders, 5th Edition, Text-Revision (DSM-5-TR) does not use the term “substance dependence” in its characterization of Substance Use Disorder (SUD), but refers to papers that do (American Psychiatric Association, 2013; McLellen et. al., 2000). That being said, two of the DSM-5-TR’s symptoms for SUD, tolerance and withdrawal, are based on a concept of dependence defined by previous DSM editions.

Indeed, dependence and addiction are *ibid* in various editions of the DSM. According to the DSM-5-TR, “substance use disorder” (SUD) is a straightforward bundling of dependence-related criteria and abuse criteria. To understand the significance of such an *ibid*, we should look back to the start of the DSM. Addiction has not *always* been associated with dependence. Like many psychiatric concepts, the definition of addiction has changed considerably over time, and to this day, there is no agreed upon definition of it (Acker, 1991; Robinson and Adinoff, 2016).

Roughly 70 years ago, in 1952, the first DSM characterized “addiction” as a mental disorder that was “part of the Sociopathic Personality Disturbance [cluster]. Individuals in this group of personality disorders were described as ‘ill primarily in terms of society and of conformity with the prevailing cultural milieu’” (Norko and Fitch, 2014). It was not until 16 years later, with the DSM-II, that addiction became explicitly associated with substances and dependence. Although still a personality disorder, this new manual used the term “drug dependence” to describe the disorder, and addiction was measured in terms of withdrawal symptoms (Acker, 2016). However, the introduction of dependence-as-withdrawal into the definition of addiction created diagnostic contradictions for psychiatrists: To keep socially

acceptable dependencies, like the regular use of prescription medication, non-pathological, the DSM would have to exclude some substances from the diagnosis of addiction ad hoc. The next two editions tried to offer a distinction between dependence and abuse, but both were still considered pathological forms of use. Then, in 2013, the DSM-5 again merged the two categories. With the DSM-5-TR, we see “substance use disorder” as a bundling of the former dependence-criteria and substance abuse criteria. Now that abuse and dependence describe the same disorder, and “dependence” is sometimes used as a synonym for “addiction,” the symptoms of abuse have become associated with discussions about dependence. Ultimately, we are left with the same problem that we had back with the DSM-II: the DSM-5 will have had to exclude some substances from the diagnosis of addiction ad hoc. The DSM-5-TR explicitly excludes caffeine—and only caffeine—from the general diagnosis of SUD.

Hopefully by now my worry is starting to become clear. If dependence implies subjugation, then my dependence on caffeine is prudentially bad for me. My relationship with caffeine somehow limits my agency, or perhaps I am controlled by it. At the same time, it would be strange to think that every person who depends on their cup of coffee to start the day is thereby pathological. Cultural resources like the DSM are at pains to explicitly delegitimize the link between caffeine-use and addiction. In other words, it is still an open question whether being in a dependence-like relation is necessarily pathological. We might be ‘getting it wrong’ by misdiagnosing possible addictions and thereby acting paternalistically, or otherwise harmfully. In particular, we should remember that being an addict can be a significant aspect of one’s identity. To misdiagnose someone with addiction is therefore not only ethically harmful insofar as patients are misled, but also counter-productive if the goal is the patient’s welfare.

Dependence and addiction are importantly distinct in ways that are occluded by the oft-presumed interchangeability of those terms in the psychiatric context—or so I will argue. In order to substantiate this claim, we must understand in what sense a dependent is limited or controlled by the thing they depend on. In the addiction literature in particular, this is often cached out in terms of ‘compulsion.’ Unpacking ‘compulsion’ is the task of the next section.

4. Picking Apart Compulsion

There are many resources on addiction outside of the DSMs, and they share a common thread. Namely, addiction has been widely characterized as a compulsive disorder (Gul and Pesendorfer, 2007). According to the National Institute on Drug Abuse (NIDA) and the World Health Organization (WHO), “Addiction is widely viewed as a chronic, relapsing, neurobiological disease characterized by compulsive use of drugs or alcohol” (Pickard, 2012). There are two interpretations of the word “compulsive” that are operative here. Sometimes, compulsion can refer to ‘irresistibility.’ On the other hand, compulsion may be interpreted to mean: acting in an *akratic* way, or contrary to one’s best judgment.

The Irresistibility Model

Consider compulsion understood to mean irresistibility. This is the conception of addiction dismantled in Pickard’s (2018) “The Puzzle of Addiction.” Irresistibility refers to the loss of autonomy. If a drug is irresistible, then one literally cannot stop herself from consuming it. In the case of addiction, behavior is said to be “uncontrollable,” or “a response to an uncontrollable drive or desire” (Hirschman, 1992; Miller, 2009). Leshner (2001) says, “the essence of addiction:

uncontrollable, compulsive drug craving, seeking, and use.” It is not just substance addiction that receives this characterization. Addiction as irresistibility prevails in other literature on addiction as well. For instance, in 2004, the director of the District of Columbia Bar Lawyer Counseling Program, Lynn Phillips, defines a sexual addiction as “a compulsive dependence on any sexual behavior that preoccupies the addict, who continues to act compulsively...”. Psychologists Tang et. al.’s (2017) and Rumpf et. al.’s (2014) papers on internet addiction describe addiction as uncontrollable as well. Physical activity and healthy aging experts, Hausenblas et. al. (2017), subscribe to a similar irresistibility model, calling addiction to exercise an “obligation.”

Returning to the DSM-5-TR, at least three of its 11 symptoms for substance use disorder—two of which are enough to diagnose mild substance use disorder—are based on the irresistibility model.

- 1) “The substance is often taken in larger amounts or over a longer period than was intended.”
- 2) “There is a persistent desire or unsuccessful effort to cut down or control use of the substance.”
- 3) “Craving, or a strong desire or urge to use the substance, occurs.”

(American Psychiatric Association, 2013)

For example, experiencing craving and use longer than intended are enough to diagnose mild SUD. The apparent ease at which at least mild SUD is diagnosable is why the definition of compulsion, as it exists relative to addiction, matters. The DSM-5-TR evidently partially adopted the irresistibility model.

We need to address two concerns about the characterization of addiction as irresistibility. First and foremost, it is possible for the alcoholic not to drink, for the gambling addict not to gamble, and so on (Pickard, 2012, 2018). Of course, addicts can be forcibly separated from the object of addiction through a lack of access; but that is not what I am referring to. In fact, addicts, incentivized appropriately, will choose to forgo possible engagement with their addictions in pursuit of more enticing ends (Pickard, 2012, 2018). So, clearly, the word “irresistible” should not be taken literally. Following philosopher Jay Wallace (1999), and further corroborated in Pickard (2018), any viable concept of the loss of will in addiction necessarily maintains some sense of volitional capacity—thus a total loss of autonomy is not viable.

Perhaps the irresistible nature of a compulsion can be understood as “driven” rather than referring to the wholesale loss of autonomy. Instead of implying necessity, to be ‘irresistibly driven’ could mean to be ‘uncontrollably motivated.’ Uncontrollable motivation refers to a motivating mental state that arises uncontrollably. For example, one might say that Sam is an addict because Sam is uncontrollably motivated to gamble. Motivation to gamble that arises uncontrollably does not necessarily mean that Sam will gamble. In other words, even if one is motivated to gamble, she may not actually pursue gambling. Let us call this the *uncontrollable drives model*.

Suppose Walter is an addict. According to the uncontrollable drives model, his uncontrollable drives are causing him clinically significant distress. The uncontrollable drives model accommodates the idea that some autonomy is lost in cases of compulsive acts insofar as one’s autonomy extends to long-term benefits. In alcohol addiction, specifically, we could say that one struggles to integrate alcohol into one’s long-term plans (Corrado, 1999; Levy, 2006). Indeed, the same logic applies to any addiction: the gambling fails to integrate into long term

plans, or shopping fails to integrate into long-term plans, and so on. As philosopher Michael Louis Corrado writes in his analysis of the treatment of addicts by the justice system,

What a defect of will must mean, if it is to serve as an excuse, must be something like this: the actor's choice is inconsistent with his ordered preferences. The actor who does something that is against his higher desires acts "against his will," so to speak, and thus unwillingly. (Corrado, 1999)

Corrado's suggestion maintains the sense of self-defeating behavior that characterizes addiction, without relying on the compromise of autonomy wholesale. Rather, the presence of a strongly motivating mental state makes it less likely that one achieves their long term goals of sobriety. In fact, I am sympathetic to Corrado's characterization of addiction here. More generally, we may understand the uncontrollable drives model as explaining the dependent person's behavior as 'compulsive' insofar as it is driven towards ends they ultimately reject. I realize that we are far from the original interpretation of 'irresistibility' as the focus shifts to values. I will say more about this move in the next section.

The Akratic Model

Akasia is not a term typically found in clinical literature. It is a philosophical term referred to in self-control literature that refers to the state of weak will one is in when intentionally performing an act that is contrary to one's best judgment (Mele, 1996). The *akratic model*, then, refers to the understanding of 'compulsive' behavior as action contrary to one's best judgment. For example,

a person might believe that she should not drink, and choose to drink anyway—she ‘changed her mind.’ In this case, we might describe her as ‘compulsive’ or weak-willed. This is not to deny that her judgment was ‘I should drink’ at the time she drank, but rather to point out that here switching back and forth is in tension with some overall assessment she has of how she should be behaving over time.

Philosopher Neil Levy (2006) argues that we should understand cases like these in terms of “oscillating preferences.” He writes,

Most of the time, the addict sincerely disavows her addiction and wishes to be rid of it.

But she regularly changes her mind; when she does, she genuinely prefers consumption to abstention. (Levy, 2006)

Like the uncontrollable drives version of the irresistibility model, the akratic model *also* explains the dependent person’s behavior as ‘compulsive’ insofar as it is driven towards ends they ultimately reject. I argue that the two models in fact collapse. The psychological apparatus required for explaining the sense in which a dependent person behaves compulsively is the same in each case, namely, she has an overall assessment according to which she should behave in one way, and a persistent motivation to do the opposite. This is the sense in which ‘Sam cannot stop gambling.’ Of course, this raises an important question, namely, *what role does a person’s own judgment about their goals, values, or over all self-interest play in determining whether or not they are dependent?* I will return to this issue in the next section.

In all interpretations of the compulsion-based model, we can see the evaluatively negative interpretation of dependence. One is subordinate to, or enslaved by, what one is

irresistibly compelled to pursue, or irresistibly motivated to pursue, or otherwise driven to pursue contrary to the actor's best-judgment. However, this analysis suggests that what *really* matters in all three cases is that there is a mismatch between one's values and outcomes. In other words, when we try to see what is valuable in the notion of "addiction as dependence" or "addiction as compulsion," the point that ends up mattering is that there is a mismatch between one's values and one's reality.

So far I have argued that the notion of dependence operative in discussions of addiction is grounded in two importantly similar models of compulsion. In short, the dependent person is limited or controlled by the thing they depend on inasmuch as they are psychologically driven to act in a certain way with respect to that thing. Dependence becomes problematic when drives run contrary to interests.

Before transitioning to my positive argument, I should emphasize two key points. First, I should reiterate that dependence does not confer irresistibility in the 'wholesale loss of agency sense.' Second, and more importantly, dependence comes apart from pathology. I will address these points in order.

One can depend on Ibuprofen to relieve pain, but may not *need* it in order to relieve the pain; perhaps Tylenol would be sufficient. Since dependence does not always confer necessity in this way, to say that dependence implies *need* is misleading. To take another example, one can say that she depends on her computer to write essays; and, assuming her computer is her *only* way of writing essays, she thereby *needs* her computer in order to write essays. It would thus be accurate to say that she is currently dependent on her computer, or even that she needs it—a need which is outside her control due to contingent circumstance—but inaccurate to say that she is

irresistibly compelled to use her computer. For instance, she may decide to fail the class. All that is ‘needed’ is to successfully achieve one's goals.

Of course, on the uncontrollable drives model, motivation itself is characterized as uncontrollable. This is actually quite common—not criteria enough for pathology. Consider your obvious dependence on water. We did not choose to need water. We cannot stop drinking it if we want to be alive, and we have cravings for it, known as thirst. Thirst is an excellent candidate for an uncontrollable drive. As Carl Elliott expresses this claim, an addict "must go where the addiction leads her, because the addiction holds the leash" (as cited in Pickard, 2018). Water holds our leashes. Most people drink multiple helpings of water every day for most of their lives. If not for the ad hoc manner in which substances are considered legitimate candidates for addiction, I would think that our relationship with water would get flagged as SUD.

One might ask, why is it not then? According to Robert West, a writer for the European Monitoring Centre for Drugs and Drug Addiction, it is because water has survival value. Robert West defined addiction in line with the uncontrollable drive model, but with a twist. He defined it as “a repeated powerful motivation to engage in a purposeful behavior that has no survival value, acquired as a result of engaging in that behavior, with significant potential for unintended harm” (as cited in Clark-Doane and Tabb, 2022). Water might be an easy case for West. One might ask whether any substance has survival value, and get various responses. Survival value quickly gets confusing to cache out from a clinical perspective.⁴ Clearly, whatever someone means by “alcohol-dependent” (when conflated with addiction) is not the same kind of judgment enjoyed

⁴There is significant literature on the role of well-being judgments in diagnosis (cf: Washington, 2018; Knox, 2023).

by the phrase “water-dependent”. Water might hold my leash, but that’s not a bad thing. Some additional negative evaluation is required to justify alcohol dependence as a pathology.

5. Addicts Act Against Their Goals

So far I have referred to dependence on illicit drugs versus dependence on medication, on alcohol, on water, and so on. What has been evident from the discussion is that dependence is best understood as a tertiary relation between the dependent, the depended, and the reason or goal to be achieved. Simply put: For intentional agents, the format of dependence that I argue is most useful is structured as follows: *A depends on X to achieve G* (where all A’s are agents). We should understand any dependence claims in psychiatric contexts using this evaluatively neutral teleological structure.

One could say that she depends on a skateboard to commute to work, or that she depends on drugs to reduce pain. There is no prudential difference between those two instances in terms of the neutral structure of dependence; both are equally examples of *A depends on X to achieve G*. With this way of discussing normal, everyday, dependence, we can discuss pathological use by way of “unsuccessful dependence.” Indeed one is not always successful at achieving one’s goal; and persistent dependence on that which undermines goal-achievement is how I believe the current model characterizes addiction. One might try to depend on a knife to cut through carrots, but it might not cut through the carrot. One might try to depend on a knife to cut through carrots, and it might do it, but it might cause such awful side effects, like the removal of a finger, that she ultimately regrets the experience. One might depend on a knife to cut through carrots and simply get what she wants, no hassle. Or, one might try to depend on a knife to cut through carrots, and undergo acceptable, but nevertheless present, costs—like the purchasing of the knife. We need to

therefore make a few formal distinctions before applying the neutral structure of dependence to addiction diagnosis.

First there is the distinction between “successful dependencies” and “unsuccessful dependencies.” A successful dependence is when A actually achieves G through X, without unacceptable associated consequences. Unsuccessful dependence, by contrast, is failure either in terms of those unacceptable consequences or in terms of failed goal pursuit: where A does not achieve G or the version of G that she wanted (through X, and without other unaimed-for consequences that undercut the satisfactory achievement of G). There are thus *two* ways a dependence can fail: directly or indirectly.

Direct failure is the most basic of the two kinds of failure: when A does not achieve G through X in any way. For example, imagine Carl depends on tobacco for relaxation. Carl smokes tobacco to relax for a few weeks—gives it a ‘good college try’—and finds that he is still not relaxed enough after each cigarette. We would then be able to say that Carl’s dependence on tobacco for relaxation is unsuccessful, on account of direct failure. He simply does not achieve his goal. Were he to find himself instead pleased with his state of mind upon smoking tobacco (consequences aside), we would say that Carl’s dependence on tobacco for relaxation was successful. Similarly, I drink water to stay hydrated, and I get exactly what I want.

Indirect failure is more complicated. Indirect failure stands in opposition to the “without unacceptable associated consequences.” Thus, an *indirectly unsuccessful dependence* is when achieving G through X causes associated consequences that undermine the original pursuit. In other words, it is when side-effects are too burdensome to bear. The aimed-for outcome is no longer worth what it has taken to get there. Indirect failure is more complicated than direct failure because assessing for indirect failure requires different outcomes to be commensurable in

a way that allows us to have a hierarchy of priorities—value-judgements about them. For example, Alice might have a directly successful dependence on certain medication to relieve symptoms of depression, but find additionally that the medication makes her so anxious and jittery that she is arguably worse-off than she was before the medication. However, the keyword there is “arguably.” It is not always an easy calculation to decide that something’s side-effects are heavier than its benefits. Alice may judge that the medication relieved depression, but is a bad fit for Alice’s life overall. So, as two general rules: 1) Achieving the goal (direct success) is a prerequisite for indirect failure, and 2) indirect failure trumps direct success.

There are, on the surface, two versions of dependence that people refer to when they are talking about dependence and addiction at the same time: physiological dependence and psychological dependence. At this point, let’s apply the structure of dependence to those phenomena in order to demonstrate how the structure works.

First, consider physiological dependence. The signs of physiological dependence, relevant to SUD (according to the DSM-5-TR), are tolerance and withdrawal. In this context, withdrawal is to feel negative physiological effects after abstention from substance-use (usually, a substance that one had used regularly). So, for instance, the regular alcohol user might abstain from alcohol-use and experience physical symptoms of withdrawal, such as: body aches and fatigue. Oftentimes, one may bring up withdrawal symptoms in a conversation about addiction in order to explain that the alcohol-user (for instance) uses alcohol to manage withdrawal symptoms caused by sobriety. So, in terms of the neutral structure of dependence—The alcohol-user (A) depends on alcohol (X) to reduce physical withdrawal symptoms (G). One could be physiologically dependent on alcohol in that sense.

Psychological dependence follows the same structure. For instance, the regular alcohol user might abstain from alcohol-use and experience psychological symptoms of withdrawal, such as irritability and mood-swings. Then the re-introduction of alcohol might reduce those symptoms. So, the alcohol-user (A) depends on alcohol (X) to reduce psychological withdrawal symptoms (G). One could be psychologically dependent on alcohol in that sense.

As I have argued, we currently exist in a cultural context in which many people are eager to interpret all dependencies as subjugations—even one's need for water. This applies to both examples above. However, referring to the neutral structure of dependence, those instances of subjugation do not necessarily confer pathologies. Sometimes one's constant engagement with a substance is pathological and sometimes it is not. I argue that pathology occurs when attempted dependence is persistently unsuccessful. This is the missing evaluative component referred to above.

People do not generally think of our dependencies on water to survive as addictions to water because we are successfully dependent on water—we approve of the goal, we successfully achieve the goal, and we accept the consequences when we deploy the strategy. There are situations wherein we do not think of prescribed use of drugs as an addiction, even if one becomes extremely dependent on the drugs, for the same reasons. That is, people use prescribed drugs to achieve a certain desirable end, and achieve said end without unacceptable side-effects. Such conclusions align perfectly with Levine and Troiden's (1988) and Pienaar et. al's. (2017) positions that even heavy drug-use can be non-pathological—or at least not an addiction.

Conversely, the kind of dependence being referred to when “alcohol dependence” means “alcohol addiction” is now perhaps better understood as a kind of “unsuccessful dependence.” Even if the alcoholic gets what she wants, directly, from drinking—e.g. relaxation—she is still

considered an alcoholic if the costs from drinking—e.g. loss of job/relationship/health—out-weigh the benefits (i.e. indirectly unsuccessful dependence). On this account, it would no longer make sense to call alcoholics necessarily “alcohol-dependent” because that is the issue—they cannot depend on alcohol (successfully).

6. Conclusion

With our newfound understanding of successful dependence, let’s return to the question: *why did the DSM-5-TR exclude caffeine from the general diagnosis of SUD?* According to a faculty member from the Center of Addiction Research, Meredith Addicott:

The DSM-5 does not include a diagnosis of caffeine use disorder (CUD) because, according [to] the APA, it is not yet clear to what extent it is a clinically significant disorder... [A] higher threshold is intended to prevent over-diagnosis of CUD given the prevalence of nonproblematic caffeine use in the general population. (Addicott, 2014)

What does Addicott mean by “nonproblematic caffeine use” (Addicott 2014)? I posit that “nonproblematic caffeine use” is when caffeine-users are successfully dependent on caffeine for social and productive lives. The sacrifices people are willing to endure increase in proportion to the amount that social and productive lives matter—which tends to be a significant amount. Being willing to bear a heavy cost to attain a certain successful dependence does not make that cost clinically significant distress. Is a social and productive life worth having to deal with tolerance, withdrawal, craving, taking larger amounts or over a longer period than was intended, and persistent desire or unsuccessful effort to cut down or control use? Those are five symptoms

of SUD that together would qualify for moderate addiction in the case of alcohol or heroin, likely because they would instill much greater distress. However, additionally, a social and productive life is widely agreed upon as a valuable goal, possibly worth the five mentioned symptoms, acknowledging the associated distress. So, the DSM-5-TR excludes caffeine from the general diagnosis of SUD and seeks to develop a higher threshold criteria for caffeine use disorder. In other words, even if it is only for caffeine, the DSM-5-TR recognizes that successful dependence is not addiction.

Lastly, let us return again to the “willing addict,” described in section two. According to the PUD model, the only way a “willing” user can still be called an addict is if her alleged “willingness” is not representative of her actual goals—that is, if she is self-deceived in some capacity. If she is in fact lucidly representing her values, if she is authentically willing to keep drinking alcohol in light of the associated consequences, then her use of alcohol is no more pathological than our everyday use of water. It would not be pathological use. This seems to be a case where the fact that the individual is not an addict is meaningful. It cannot simply be the case that every time someone uses a substance in a way that does not align with social values that it is a case of addiction. We want to be able to identify individuals in society whose values differ from our own. Consider an analogy: The mass-murderer is not addicted to guns for using a gun to kill everyone. However, they may be mentally ill for wanting to kill everyone. The diagnosis of mental illness on that account would require a normative understanding of the specific value being held and achieved—namely, mass-murder. That is, we think it is bad to have that value, so we call mass-murderers mentally ill. Such an account is different from thinking that undermining one’s own goals is criteria for mental illness. The mass-murderer is not undermining her goals. Similarly, the willing user is not undermining her goals. If the willing user genuinely wants to

lose her friends, family, and hobbies—then she is not being an addict to do so. Nevertheless, she may have a serious psychiatric condition. So, should we want to “treat” or “help” someone like this, one would follow the incorrect protocol to diagnose her with addiction.

In this paper, I argued that conflating dependence with addiction is a misleading and harmful practice. I offered a way to analyze dependence such that referring to it across contexts maintains greater consistency. Of course, more work is required to properly defend my view of addiction based on the analysis of dependence I offered here. For instance, how exactly do prudential values inform indirectly unsuccessful dependencies? How plausible is self-deception in addiction? How does the PUD model handle cases of “adaptive preference,” as in philosopher Anita Superson’s (2005) case of the “deferential wife,” or Serene Khader’s (2011) deliberative perfectionist approach? With these questions on the horizon, what I offer here is the groundwork for my view. Separating addiction from successful dependence will add to the literature on addiction because, up until recently, addiction has often been positively associated with dependence, arguably being treated as a synonym for it. We should flip the script, so to speak. Dependence and addiction are more like antonyms than synonyms.

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